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F-18 FDG PET-CT SCAN (WHOLE BODY)

Patient:	**MRS. ROSALIND M. FERNANDES**	PET ID:	**AINM/PET/26/0388**
Age / Sex:	48 Y / F	Study date:	**3rd March 2026**
Referred by:	Dr. Anselm Pereira (Pulmonology)	Tracer:	F-18 FDG 7.8 mCi; fasting BSL 96 mg/dL

Clinical history: Non-smoker. Cough and left-sided chest pain x 2 months. CECT thorax (18/02/2026): left upper lobe mass. CT-guided biopsy HPE (report dated 26/02/2026): adenocarcinoma. Referred for staging.

Technique

Images acquired from the vertex to mid-thigh 60 minutes after intravenous FDG, with low-dose non-contrast CT for attenuation correction and anatomical localisation.

Findings

- Thorax: FDG-avid spiculated mass 4.6 x 3.9 cm in the apico-posterior segment of the left upper lobe (SUVmax 11.8). FDG-avid left hilar (SUVmax 7.2), subaortic and para-aortic (station 5 and 6) nodes, largest 1.8 cm. Small left pleural effusion with a 9 mm avid pleural nodule (SUVmax 4.1).
- Skeleton: FDG-avid lytic lesions in the D9 vertebral body (SUVmax 8.4), the right iliac bone and the left 6th rib.
- Liver, adrenals, spleen and abdominal nodes: no abnormal FDG uptake.
- Brain: physiological cortical uptake limits evaluation; MRI brain with contrast is suggested.

Impression

FDG-avid left upper lobe mass with left hilar and mediastinal (station 5 and 6) nodal disease, a left pleural deposit with effusion and skeletal metastases (D9, right ilium, left 6th rib), consistent with a metastatic primary lung malignancy.

Dr. Clinton Rodrigues
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Reg. No. GMC-3916
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*** End of Report ***

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